



Health Insurance Guide

BluePeak Technologies | 2026 medical coverage

Document ID	BPT-HR-MED-111	Version	v1.0
Effective date	2026-01-01	Next review	2026-10-01
Owner	Total Rewards	Approved by	Benefits Committee
Classification	Internal	Category	Benefits
Audience	All employees	Status	Approved
Source	https://employee.bluepeaktech.com/resource/s/benefits/health-insurance	Access scope	All employees

Document purpose. 2026 medical-plan eligibility, enrollment, premiums, plan comparisons, HSA and FSA coordination, prescriptions, telehealth, life events, claims, and coverage continuation.

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2026 Plan Snapshot

Feature	Core HDHP	Select PPO	Premier PPO
HSA eligible	Yes	No	No
In-network deductible - individual/family	\$1,800 / \$3,600	\$750 / \$1,500	\$350 / \$700
In-network out-of-pocket maximum - individual/family	\$3,600 / \$7,200	\$4,000 / \$8,000	\$3,000 / \$6,000
Primary care	20% after deductible	\$30 copay	\$20 copay
Specialist	20% after deductible	\$50 copay	\$35 copay
Urgent care	20% after deductible	\$75 copay	\$50 copay
Emergency room	20% after deductible	\$300 plus 20%	\$250 plus 10%
Company HSA contribution	\$750 employee-only; \$1,500 other tiers	Not applicable	Not applicable

Plan document rule: This guide summarizes the 2026 program. The official plan documents, insurance certificate, network rules, and applicable law control claims, exclusions, medical necessity, appeals, and coverage rights.

1. Eligibility and Coverage Start

Regular U.S. employees scheduled at least 20 hours per week are eligible for medical coverage. Eligible dependents include a legal spouse, eligible domestic partner, and children through the end of the month in which they reach age 26, plus older disabled dependents who meet plan requirements.

New-hire elections are due within 30 calendar days of the start date. Coverage begins on the first day of the month on or after the start date when the election is completed on time. An employee who starts on the first day of a month may have coverage effective that day.

Employees scheduled fewer than 20 hours, contractors, and temporary workers are not eligible unless written terms or law state otherwise.

1.1 Dublin Employees

Dublin employees participate in the BluePeak Ireland Private Medical Plan and statutory system rather than the U.S. plans. BluePeak pays 90 percent of employee-only private medical premium and 70 percent of eligible dependent premium. The local member booklet controls coverage and claims.

2. Enrollment and Waiver

Enroll through the Benefits Portal using Microsoft Entra ID. Review each election, covered dependent, tobacco-status question where lawful, HSA or FSA election, and beneficiary information. A waiver confirms that the employee declines BluePeak medical coverage for the plan year; it does not waive the right to enroll after a qualifying life event.

Dependent verification must be uploaded through the secure portal. Acceptable documents are listed in the workflow. Do not email Social Security numbers, birth certificates, marriage records, or medical information.

3. Employee Medical Premiums

Premiums are deducted semi-monthly before tax when permitted. Domestic-partner coverage may create imputed taxable income.

Coverage tier	Core HDHP	Select PPO	Premier PPO
Employee only	\$48	\$84	\$139
Employee + spouse/domestic partner	\$126	\$204	\$326

Coverage tier	Core HDHP	Select PPO	Premier PPO
Employee + child(ren)	\$108	\$178	\$284
Family	\$188	\$310	\$468

Rates are per semi-monthly paycheck. Employees on an unpaid leave must arrange payment to avoid cancellation. BluePeak may adjust premiums at the start of a plan year after open enrollment notice.

4. Choosing a Plan

4.1 Core HDHP

The Core HDHP generally has the lowest payroll premium and the highest upfront deductible. With limited exceptions for preventive care, the employee pays the negotiated cost until the deductible is met, then pays 20 percent in network. The plan is HSA eligible. It may suit employees who want lower premiums, can fund the deductible, and value HSA tax advantages.

4.2 Select PPO

The Select PPO uses copays for many office and urgent-care services and a moderate deductible for other services. It may suit employees who prefer predictable visit costs or expect regular care and prescriptions.

4.3 Premier PPO

The Premier PPO has the highest payroll premium, lowest deductible, and lower copays and coinsurance. It may suit employees who expect substantial use or prefer lower point-of-service cost, but total annual cost depends on actual care.

No plan is best for every employee. Compare payroll premiums, expected services, prescriptions, provider network, deductible exposure, HSA value, and family needs.

5. Covered Services and Cost Sharing

All plans cover preventive care in network at 100 percent when coded as preventive under plan rules. Diagnostic services performed during a preventive visit may be subject to deductible, copay, or coinsurance.

Service	Core HDHP	Select PPO	Premier PPO
Preventive visit	\$0 in network	\$0 in network	\$0 in network
Primary care	20% after deductible	\$30	\$20
Specialist	20% after deductible	\$50	\$35
Outpatient mental-health visit	20% after deductible	\$30	\$20
Urgent care	20% after deductible	\$75	\$50
Emergency room	20% after deductible	\$300 + 20%	\$250 + 10%
Inpatient hospital	20% after deductible	20% after deductible	10% after deductible
Diagnostic imaging	20% after deductible	20% after deductible	10% after deductible
Telehealth general medicine	20% after deductible	\$20	\$10

Out-of-network care is subject to a separate, higher deductible and coinsurance, balance billing, and plan limitations. Emergency care is handled under emergency-service rules. Confirm network status with the plan before non-emergency care.

6. Prescription Drug Coverage

The prescription program uses a formulary and participating pharmacies. Prior authorization, step therapy, quantity limits, and specialty-pharmacy rules may apply.

30-day in-network retail tier	Core HDHP	Select PPO	Premier PPO
Preventive formulary drugs	Plan rules; some at \$0	\$0-\$10	\$0-\$10
Generic	After deductible, then \$15	\$15	\$10
Preferred brand	After deductible, then \$45	\$45	\$35
Non-preferred brand	After deductible, then \$75	\$75	\$60
Specialty	After deductible, then 25% up to plan maximum	25% up to plan maximum	20% up to plan maximum

Mail-order and 90-day retail options may reduce cost. The formulary can change during the year with required notice.

7. Health Savings Account

The HSA is available only with the Core HDHP and only when the employee meets federal eligibility requirements. An employee generally cannot contribute to an HSA while enrolled in non-HDHP medical coverage, a general-purpose health FSA, or certain other coverage.

BluePeak contributes \$750 annually for employee-only coverage and \$1,500 for employee-plus-dependent coverage, deposited pro rata each pay period after the HSA is opened. New-hire contributions are prorated. The employee owns the account and keeps the balance after leaving BluePeak.

Employee contributions may be changed during the year through payroll, subject to annual legal limits. HSA funds may be used for qualified health expenses; the employee is responsible for tax eligibility and records.

8. Flexible Spending Accounts

Employees who do not enroll in the Core HDHP with an HSA may elect a health care FSA. A limited-purpose FSA may be available with an HSA for eligible dental and vision expenses. A dependent care FSA may reimburse eligible care expenses that allow the employee and spouse, if applicable, to work or seek work.

Annual legal limits and carryover amounts are displayed during enrollment because they may change. FSA elections are generally fixed for the plan year unless a qualifying life event permits a change.

9. Open Enrollment

The 2027 open enrollment window is scheduled for November 2 through November 16, 2026. Elections take effect January 1, 2027. Benefits sends plan and rate information before the window. Employees should actively review elections even when the system allows passive rollover; HSA and FSA contribution elections may require a new annual election.

10. Qualifying Life Events

An employee may change elections within 30 calendar days after a qualifying event, including marriage, divorce, birth, adoption, loss or gain of other coverage, or an eligible change in dependent status. Certain Medicaid or CHIP events may have a different legal window.

The requested change must be consistent with the event. Coverage for a newborn or newly adopted child may be retroactive to the event date when the request is timely. Submit the event in the Benefits Portal even when supporting documentation is not immediately available.

11. Finding Care and Preauthorization

Use the Benefits Portal link to the medical administrator's current provider directory. Confirm network status with both the facility and individual clinician. A provider's statement that it "accepts" the plan does not always mean the provider is in network.

Certain inpatient, surgical, imaging, specialty-drug, therapy, and durable-medical-equipment services require preauthorization. The member remains responsible for following plan requirements even when the provider assists.

12. Claims, Explanations of Benefits, and Appeals

An Explanation of Benefits is not a bill. Compare it with the provider bill and contact the plan for errors. Claims and appeals follow the deadlines and address in the official plan documents. Benefits may help explain the process but does not decide medical necessity or overturn an administrator's claim decision.

Report a suspected billing, identity, or privacy issue promptly. Do not send full medical records to BluePeak unless the secure benefits process specifically requests them.

13. Leave, Reduced Hours, and Coverage Continuation

Coverage continues during paid leave under normal payroll deductions. During an approved unpaid leave, the employee must pay the required contribution by the stated deadline. Failure to pay after required notice may end coverage.

U.S. employees who lose coverage because of a qualifying event may be offered continuation coverage under COBRA or an applicable state program. The election notice, cost, and deadlines are provided by the administrator. Separation coverage normally ends on the last day of the month in which employment ends, unless a plan or law states otherwise.

14. Coordination of Benefits and Medicare

Employees covered by more than one plan must provide coordination information. The plans determine primary and secondary responsibility under their rules. Employees approaching Medicare eligibility should consult Medicare and the Benefits team before changing coverage; BluePeak cannot provide personal tax or Medicare advice.

15. Frequently Asked Questions

15.1 Which plan includes an HSA?

Only the Core HDHP.

15.2 What is the Premier family premium?

\$468 per semi-monthly paycheck for the 2026 plan year.

15.3 When can I add a new baby?

Submit the qualifying life event within 30 calendar days of birth. Timely enrollment can make coverage effective on the birth date.

15.4 Are all preventive services free?

Covered preventive services are \$0 in network when they meet plan and coding rules. Diagnostic services may have cost sharing.

15.5 Who can see my claims?

The plan administrator and providers handle claims. BluePeak receives only information permitted for plan administration and protects it with restricted access.

90. Administration, Exceptions, and Questions

BluePeak administers medical benefits consistently with the employee's work location, employment classification, written employment terms, and applicable law. A statutory or contractual entitlement that is more generous than this document will apply to the extent required. A manager may not waive a legal requirement, create a new benefit, or promise an exception without written approval from the document owner.

Employees should raise questions before relying on an informal interpretation. Contact benefits@bluepeaktech.com through Outlook or the People Help channel in Microsoft Teams. Payroll, Legal, Information Security, or a site leader will be consulted when the question involves tax, wage-and-hour, privacy, immigration, safety, or local-law considerations.

Approved exceptions must identify the employee, the policy provision, the business reason, the approving authority, the start and end date, and any conditions. Exceptions are stored with the employee or transaction record and do not establish a precedent for other employees.

91. Related Documents and Systems

Resource	When to use it
Benefits Enrollment Form	Medical elections and dependent enrollment when HR directs form use.
Dental & Vision Benefits	Dental and vision plan designs, premiums, copays, and allowances.
Life & Disability Insurance	Life, AD&D, short-term disability, and long-term disability.
Employee Wellness Program	EAP, fitness reimbursement, caregiving, and wellbeing resources.
Parental Leave Policy	Benefit continuation and qualifying life events after birth or placement.

Revision History

Version	Effective date	Change summary
v1.0	2026-01-01	Initial standalone 2026 medical guide split from the combined health and welfare guide.